

Dear [PROVIDER-NAME],

Re Ms [NAME]; [NHS-NO]; [DATE]; [ADDRESS], Portswood, Southampton, [POSTCODE]

[ADDRESS]

curative chemotherapy on the [DATE]. She has responded well to treatment. I write to summarise response so far.

Initial Staging & Diagnosis

Ms. [NAME] was initially diagnosed in January with FIGO stage III endometrial adenocarcinoma. diagnosis was confirmed via biopsy, and subsequent staging was determined based on imaging and histopathological evaluation.

Histological analysis confirmed a high-grade endometrioid adenocarcinoma. Genetic testing revealed microsatellite instability (MSI-High) and the presence of a POLE mutation, which is often associated with a favourable prognosis and higher sensitivity to chemotherapy. HER2 status was tested and found to be negative.

Chemotherapy Regimen

On [DATE], Ms. [NAME] was commenced on a chemotherapy regimen consisting of Carboplatin (AUC 5), Paclitaxel (175 mg/m²) administered every three weeks. She has completed six cycles of this regimen with significant complications.

Response to Chemotherapy

Ms. [NAME] has shown a positive response to the chemotherapy regimen, as evidenced by the significant reduction in tumour burden noted on imaging studies. Her CA-125 levels, which were elevated at the start of treatment, have now normalized. Clinically, she has reported an improvement in symptoms.

Recent CT imaging of the chest, abdomen, and pelvis has shown a marked reduction in the size of previously noted pelvic and para-aortic lymphadenopathy. There is no evidence of new lesions or metastatic spread. The primary uterine mass has significantly decreased in size.

We will continue to monitor Ms. [NAME] with regular follow-up visits, including physical examination, blood tests, and periodic imaging studies to assess for disease recurrence. She is scheduled for MDT review.

Yours Sincerely,

[PROVIDER-NAME]

Consultant Oncologist

[ADDRESS]

[PROVIDER-NAME]

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